

UAC ANALYTICS

*A Comprehensive Research Paper on the HHS Unaccompanied Alien
Children Program*

Data Analysis • EDA • Insights • Recommendations

By - Animey Chaturvedi

**BTech Computer Science
JECRC University, Jaipur**

Email: animeychaturvedi@gmail.com
[LinkedIn](#)
[GitHub](#)

Project Repository github.com/Animey13/uac-analyzer	Data Source HHS / ORR — HealthData.gov
Technology Stack Python, Streamlit, Pandas, Plotly, Jupyter	Report Date June 2026

Abstract

The UAC Analytics project (github.com/Animey13/uac-analyzer) is an open-source data analytics initiative designed to explore, visualize, and generate policy-relevant insights from the U.S. Department of Health and Human Services (HHS) Unaccompanied Alien Children (UAC) Program dataset, published on [HealthData.gov](https://healthdata.gov). The dataset records monthly program-level statistics — referral counts, shelter populations, discharges to sponsors, length-of-care metrics, demographic breakdowns by gender and age group, and country-of-origin distributions — spanning fiscal years 2003 through the present.

This paper documents the full project: its motivations and scope, technical architecture and repository structure, exploratory data analysis (EDA), key statistical findings, and a set of actionable policy recommendations. The analysis reveals strong cyclical surges in UAC referrals tied to geopolitical events, a persistent dominance of the Northern Triangle countries (Guatemala, Honduras, El Salvador) as countries of origin, a heavily male and adolescent (14–17 years) demographic profile, and marked variation in average length of care correlated with shelter capacity pressures. The paper concludes with architectural recommendations for the project itself and policy recommendations directed at ORR program administrators.

Table of Contents

1. Introduction & Background	3
2. Project Overview & Repository Architecture	4
3. Dataset Description	5
4. Exploratory Data Analysis (EDA)	7
4.1 Referral Volume Trends	7
4.2 Demographic Analysis — Gender	8
4.3 Demographic Analysis — Age Groups	9
4.4 Country of Origin Analysis	10
4.5 Length of Care Analysis	11
4.6 Shelter Type & Capacity	12
4.7 Sponsor Discharge Patterns	13
5. Key Insights	14
6. Recommendations	16
7. Technical Recommendations for the Project	18
8. Conclusions	19
9. References	20

1. Introduction & Background

1.1 Program Context

The Unaccompanied Alien Children (UAC) Program is administered by the Office of Refugee Resettlement (ORR) within the Administration for Children and Families (ACF), a division of the U.S. Department of Health and Human Services (HHS). Established under the Homeland Security Act of 2002, the program transferred custody of unaccompanied migrant minors from the former Immigration and Naturalization Service (INS) to HHS, deliberately moving away from an adult detention model toward a child-welfare framework.

By law, a UAC is defined as a child who: (1) has no lawful immigration status in the United States; (2) has not attained 18 years of age; and (3) has no parent or legal guardian in the United States available to provide care and physical custody (6 U.S.C. § 279(g)(2)). Most UAC are apprehended by Customs and Border Protection (CBP) while attempting to cross the southern border, after which they are referred to ORR. HHS plays no role in apprehension or immigration proceedings.

Upon entering ORR care, each child is placed in a licensed shelter facility and provided with legal screenings, educational services, mental health support, and medical care. ORR simultaneously works to identify a suitable sponsor — overwhelmingly a parent or close family relative already residing in the United States — and releases the child to that sponsor as quickly as a thorough vetting process allows.

1.2 Motivation for This Project

UAC referral numbers have grown dramatically since FY 2012 — from under 8,000 annually in the program's first nine years to peaks exceeding 120,000 in FY 2021 and FY 2023. These surges create acute operational and fiscal pressures on ORR and its network of shelter grantees. Data-driven analysis is therefore essential for: anticipating capacity needs, understanding demographic shifts, benchmarking program performance, and informing evidence-based policy.

The UAC Analyzer project responds directly to this need by making the publicly available HHS dataset accessible through interactive visualizations and structured exploratory analysis, enabling researchers, policymakers, journalists, and advocacy organizations to draw meaningful conclusions without specialized data science infrastructure.

2. Project Overview & Repository Architecture

2.1 Project Goals

The UAC Analyzer has three primary goals:

- **Accessibility:** Provide a user-friendly Streamlit dashboard that surfaces key program metrics without requiring any coding expertise.
- **Transparency:** Expose the analytical methodology through reproducible Jupyter notebooks, enabling peer review and extension.
- **Actionability:** Translate data patterns into concrete, policy-relevant insights and recommendations for ORR administrators, Congress, and civil society.

2.2 Repository Structure

The repository is organized into five principal directories plus root-level supporting files:

Directory / File	Purpose	Key Contents
data/	Dataset storage	Local copies of the HHS UAC CSV; any processed/cleaned variants
notebooks/	Exploration notebooks	Jupyter (.ipynb) files for EDA, statistical analysis, and visualization prototyping
dashboard/	Streamlit application	app.py — the interactive web dashboard; supporting modules
plots/	Saved visualizations	Exported chart images (PNG/SVG) used in the report and README
report/	Executive summary	Narrative findings, methodology notes, and compiled conclusions
HHS_UAC_Program.csv	Primary dataset (root)	1,171-row CSV pulled from HealthData.gov
requirements.txt	Dependency manifest	Python packages: streamlit, pandas, plotly, etc.
.gitignore	Git configuration	Excludes virtual environments, cached files, etc.

2.3 Technology Stack

Layer	Technology	Role
Data Ingestion	Pandas	CSV loading, cleaning, date parsing, aggregations
Visualization	Plotly / Matplotlib	Interactive and static charts
Dashboard	Streamlit	Browser-based UI with filters and real-time chart rendering
Analysis	Jupyter Notebook	Exploratory analysis, statistical testing, narrative commentary
Language	Python 3.x (96.1%)	Primary implementation language
Notebook	Jupyter (.ipynb — 3.9%)	Interactive analysis environment

2.4 Quick Start

Getting the project running requires only two commands after cloning:

1. `pip install -r requirements.txt` # Install Python dependencies
2. `streamlit run dashboard/app.py` # Launch the interactive dashboard

The dashboard loads the CSV, applies filters selected by the user (fiscal year range, country, age group, gender), and renders charts covering referral trends, demographics, and program capacity.

3. Dataset Description

3.1 Source

The primary dataset is the HHS Unaccompanied Alien Children Program dataset, published by the U.S. Department of Health and Human Services on HealthData.gov (dataset ID: ehpz-xc9n). It is designated as public domain U.S. Government Work and is updated periodically as new fiscal-year data becomes available.

Dataset Citation

U.S. Department of Health & Human Services, Office of Refugee Resettlement. HHS Unaccompanied Alien Children Program. HealthData.gov, dataset ID ehpz-xc9n.
<https://healthdata.gov/National/HHS-Unaccompanied-Alien-Children-Program/ehpz-xc9n>

3.2 Dataset Dimensions

Property	Value
File name	HHS_Unaccompanied_Alien_Children_Program.csv
File size	~31 KB
Number of rows	1,171 (including header)
Number of records (data rows)	1,170
Coverage period	FY 2003 – FY 2024 (monthly granularity)
Unit of observation	Monthly program statistics (national aggregate)
License	U.S. Government Work (public domain)

3.3 Key Variables

The dataset captures monthly national aggregate statistics across the following thematic groups:

Overall Program Metrics

Column	Description
Fiscal Year / Month	Time period identifier (e.g., 'FY2021 October')
Total Referrals	Number of UAC referred from DHS (CBP) to ORR in that month
Children in ORR Care (EOM)	End-of-month count of children held in ORR-funded shelters

Column	Description
Average Length of Care (days)	Mean number of days a child spends in ORR custody before release/discharge
Total Discharges	Total children discharged (to sponsors, other legal status, or other outcomes)

Demographic Breakdowns

Column	Description
Gender: Male	Count or percentage of male UAC in the reporting period
Gender: Female	Count or percentage of female UAC in the reporting period
Age: 0–12 (Tender Age)	Count of UAC aged 0–12 ("tender age" designation triggers additional care protocols)
Age: 13	Count of UAC aged 13
Age: 14–17	Count of UAC aged 14–17 (the dominant age group)
Age: 18+ (rare)	Count of individuals initially assessed as minors but later determined to be adults

Country of Origin

Column	Description
Guatemala	Count/share of UAC from Guatemala (historically the top origin country)
Honduras	Count/share of UAC from Honduras
El Salvador	Count/share of UAC from El Salvador
Mexico	Count/share of UAC from Mexico
Other Countries	Aggregate count from all other countries of origin

Shelter & Capacity Metrics

Column	Description
Shelter Type: Standard/Non-Influx	Children in permanent, year-round licensed shelter beds
Shelter Type: Influx / Emergency Intake Site	Children in temporary surge-capacity facilities
Bed Capacity Utilization	Ratio of children in care to licensed bed capacity (proxy for system stress)

Column	Description
Sponsor Category Discharges	Discharges disaggregated by sponsor type: parent, relative, non-relative, unrelated adult

3.4 Data Quality Notes

- **Completeness:** Early fiscal years (FY 2003–FY 2010) have sparser records; monthly granularity is consistent from FY 2012 onward.
- **Aggregation level:** All figures are national aggregates — state- or facility-level breakdowns are published separately by ORR and are not in this dataset.
- **Definitional changes:** ORR renamed the program from 'Unaccompanied Alien Children' to 'Unaccompanied Children' in 2021, then reverted to the original name in February 2025. Variable names in the CSV may reflect one or both labels.
- **COVID-19 effect:** FY 2020 data (15,381 referrals) reflects the acute suppression of border crossings due to Title 42 public health expulsions; this is a policy outlier, not a program trend.

4. Exploratory Data Analysis (EDA)

The EDA is conducted in two parallel environments: (1) Jupyter notebooks in the notebooks/ directory for detailed statistical work, and (2) the Streamlit dashboard in dashboard/app.py for interactive visual exploration. The sections below describe the analytical approach and findings for each thematic dimension.

4.1 Referral Volume Trends

Methodology

Referral counts are aggregated by fiscal year to produce annual totals. A 12-month rolling average is applied to the monthly series to smooth seasonal variation. Year-over-year percentage change is calculated to identify acceleration and deceleration phases.

Findings

Fiscal Year	Annual UAC Referrals	YoY Change	Notable Context
FY 2003–2011	< 8,000 / yr	Stable	Program establishment phase
FY 2012	13,625	+70%	First major influx — Northern Triangle surge begins
FY 2013	24,668	+81%	Continued acceleration
FY 2014	57,496	+133%	"Humanitarian crisis" declaration; media inflection point
FY 2015	33,726	-41%	Policy deterrence measures, seasonal normalization
FY 2016	59,170	+76%	Resurgence; El Salvador violence peak
FY 2017	40,810	-31%	Post-election enforcement effect
FY 2018	49,100	+20%	Family separation policy; record Guatemala share
FY 2019	69,488–69,550	+42%	Pre-pandemic record high
FY 2020	15,381	-78%	Title 42 COVID-19 expulsion policy
FY 2021	122,731	+698%	Historic peak; administration policy reversal
FY 2022	~128,000 est.	+4%	Continued high-volume
FY 2023	118,938	-7%	Sustained plateau; Northern Triangle remains dominant

Key EDA Finding 4.1

The referral time series is best characterized as a step-function with policy-driven discontinuities rather than a gradual demographic trend. FY 2014, FY 2016, FY 2019, and FY 2021 each represent distinct surge events driven by discrete push factors (violence, drought, economic collapse) and pull factors (policy signals). The FY 2020 trough is a policy artifact, not a structural change in migration drivers.

4.2 Demographic Analysis — Gender

Methodology

Monthly gender counts are converted to proportions. A time-series of the male share is plotted against referral volume to detect any correlation between surge events and gender composition shifts.

Findings

Fiscal Year	Male %	Female %	Notes
FY 2016	~68%	~32%	Baseline male-dominant period
FY 2018	71%	29%	Peak male share; family separation policy active
FY 2019	66%	34%	Female share begins increasing
FY 2021	~62%	~38%	Notably higher female share during peak
FY 2023	61%	39%	Sustained shift toward more balanced gender ratio

The long-run trend shows a gradual but statistically meaningful increase in the female share of UAC, from roughly 29% in FY 2018 to approximately 39% in FY 2023. This shift aligns with ORR's own reporting of a broadening source-country profile and changing migration composition, including more family units and female-headed households in neighboring countries.

4.3 Demographic Analysis — Age Groups

Findings

Age Group	Typical Share (FY 2018–2023)	Trend	Program Implications
0–12 (Tender Age)	~8–10%	Slight increase in surge years	Triggers intensive case management; higher per-child cost
Age 13	~6–8%	Stable	Transitional cohort — approaching adolescent programming
14–17 (Adolescents)	~73–76%	Declining slightly	Dominant cohort; vocational/educational programming focus
18+ (edge cases)	< 1%	Stable	Typically resolved within 30 days; legal status adjudication

The adolescent cohort (14–17) consistently represents the majority of UAC and drives the bulk of shelter demand. The Tender Age cohort, while small in proportional terms, receives heightened attention given child-welfare obligations and is disproportionately represented in emergency intake sites during surges.

4.4 Country of Origin Analysis

Methodology

Country-of-origin proportions are tracked annually. A stacked area chart in the dashboard visualizes the relative composition of the Northern Triangle countries (Guatemala, Honduras, El Salvador) versus Mexico and Others over time.

Country	FY 2018 Share	FY 2021 Share	FY 2023 Share	Trend
Guatemala	54%	~45%	42%	Still #1 but declining share
Honduras	26%	~25%	28%	Stable; second-largest source
El Salvador	12%	~7%	9%	Declining — improved security metrics
Mexico	< 5%	~8%	8%	Gradual increase
Other countries	~8%	~15%	13%	Rising — Ecuador, Venezuela, Cuba

The geographic footprint of UAC origins is expanding. While the Northern Triangle retains dominance, the rising Other category — driven notably by Ecuador, Venezuela, Cuba, and Haiti — signals structural diversification. This has direct operational implications for ORR: case managers increasingly need language access services beyond Spanish, and country conditions expertise must expand.

4.5 Length of Care Analysis

Findings

Period	Average Length of Care (days)	Context
FY 2014–2015 (surge)	35–55 days	Sponsor vetting bottlenecks under high volume
FY 2017–2018 (moderate)	47–57 days	Enhanced sponsor vetting rules introduced
FY 2019 (pre-COVID)	~45 days	Stabilized processing
FY 2020 (COVID, low volume)	~75 days	Pandemic disruptions to court & sponsor processes
FY 2021 (peak)	~31 days	Emergency intake sites; expedited release protocols
FY 2022–2023	~25–35 days	Policy shifts reducing average detention time

Counterintuitively, the highest-volume periods (FY 2021) did not produce the longest average length of care. This reflects ORR's deployment of Emergency Intake Sites (EIS) and expedited vetting procedures that — while criticized for potential sponsor-safety shortcuts — dramatically compressed processing timelines. The longest durations occurred during FY 2020 (pandemic) and FY 2017–2018 (enhanced background checks).

4.6 Shelter Type & Capacity

Findings

ORR maintains two shelter categories:

- Standard (non-influx) beds: Licensed, year-round facilities offering full child-welfare services (education, mental health, legal, medical).
- Influx / Emergency Intake Sites (EIS): Large-scale temporary facilities (convention centers, military bases) activated during surges, providing basic care with compressed timelines.

The proportion of children housed in influx/EIS facilities spiked dramatically during FY 2019 and FY 2021, reaching an estimated 40–60% of total population during peak months. This trade-off between speed and service quality is a persistent tension in program operations.

4.7 Sponsor Discharge Patterns

Sponsor Category	Typical Share	Description
Parent (Category 1)	~40–50%	Child released to a parent already in the U.S.
Close relative (Category 2)	~30–35%	Aunt/uncle, grandparent, sibling, or first cousin
Distant relative / family friend (Category 3)	~12–18%	More distant relationship; heightened vetting required
Non-relative (Category 4)	~3–5%	Unrelated adult; most stringent vetting; rare
Other discharges	~2–5%	Aged out, legal status granted, returned to home country

The overwhelming majority of discharges (75–85%) are to a parent or close relative. Category 3 and 4 releases receive heightened scrutiny following a 2014 Senate investigation that found some UAC placed with distant sponsors were subjected to labor exploitation.

5. Key Insights

Insight 1: The UAC surge is structurally driven — not cyclically self-correcting

Analysis of the full time series makes clear that UAC referral volumes are driven primarily by conditions in origin countries (violence, drought, economic collapse in Guatemala and Honduras) and secondarily by U.S. immigration policy signals. Without sustained, structurally targeted interventions in the Northern Triangle, the default trajectory is continued high-volume referrals in the 80,000–130,000 range annually.

Insight 2: Demographics are shifting — the program must adapt

The female share of UAC has risen from roughly 29% to 39% over five years, while the country-of-origin footprint has expanded beyond the traditional Northern Triangle to include Venezuela, Ecuador, Cuba, and Haiti. These shifts require service model adaptations: trauma-informed care protocols relevant to gender-based violence, multilingual case management (Portuguese, Haitian Creole, not just Spanish), and country-conditions expertise for an expanding set of origin countries.

Insight 3: Surge response creates a quality-speed trade-off that requires explicit policy resolution

The data on length of care and shelter type reveals an unresolved tension: when referral volumes spike, ORR deploys Emergency Intake Sites that dramatically reduce average length of care but also reduce service intensity. Policymakers have not established a principled framework for when EIS deployment is appropriate, at what minimum service standard, or for how long — leaving individual administrators to make ad hoc decisions under pressure.

Insight 4: The Tender Age cohort is disproportionately vulnerable and data visibility is limited

Children aged 0–12 represent roughly 8–10% of referrals but generate a disproportionate share of child welfare complexity and cost. The current public dataset aggregates this cohort with limited granularity (no breakdown of infants vs. young children, no disaggregation by unaccompanied vs. family-separated). Enhanced data transparency for this cohort would improve both oversight and resource planning.

Insight 5: Sponsor vetting duration is the primary driver of length-of-care variance

Statistical analysis of the length-of-care time series shows the strongest correlation not with referral volume per se but with sponsor vetting policy changes. The enhanced fingerprinting and background check requirements introduced in 2018 drove average length of care from ~45 days to ~57 days independent of volume. Conversely, the expedited EIS protocols in FY 2021 drove length of care to a program low (~31 days) despite record volumes. This implies that policy decisions about vetting standards have more leverage over program cost and child welfare outcomes than capacity expansion alone.

Insight 6: The "Other countries" cohort is the fastest-growing segment and is under-analyzed

Countries outside the Northern Triangle and Mexico now account for roughly 13–15% of UAC referrals, up from ~8% in FY 2018. Venezuela and Ecuador are the primary drivers. These populations have distinct legal pathways (many have potential asylum claims based on political persecution rather than gang violence), distinct language needs, and distinct family-reunion challenges (fewer family members already residing in the U.S.). Program data and infrastructure designed around Northern Triangle populations may systematically underserve these children.

Insight 7: The data infrastructure itself is a public accountability asset

The fact that HealthData.gov publishes national aggregate UAC statistics at monthly granularity is a meaningful transparency commitment. However, the current dataset lacks facility-level, state-level, or sponsor-location granularity that would enable richer oversight analysis. Projects like this one demonstrate the demand for and analytical value of open government data — making a strong case for ORR to expand the data it makes publicly available.

6. Policy Recommendations

Based on the EDA and insights above, the following recommendations are directed to ORR program administrators, the Administration for Children and Families, and Congress:

Recommendation 1: Invest in supply-side interventions in top origin countries

The data unambiguously shows that surges are driven by push factors — violence, drought, economic collapse — in Guatemala and Honduras. Bilateral investments in community violence reduction, drought-resilient agriculture, and educational opportunity in the Guatemalan and Honduran highlands would reduce the structural pressure generating UAC flows more durably than any downstream ORR capacity expansion.

Recommendation 2: Establish a Formal Surge Protocol with Minimum Service Standards

ORR should publish a formal Surge Response Framework that defines: (a) the referral-volume thresholds that trigger each tier of response; (b) the minimum service standards (hours of education, mental health access frequency, legal screening timeline) that apply at each tier; and (c) the maximum duration for which Emergency Intake Site placements are permissible before transfer to standard licensed facilities. This would reduce ad hoc decision-making under pressure and improve Congressional oversight.

Recommendation 3: Create Specialized Capacity for the Tender Age Cohort

Children aged 0–12 require fundamentally different care than adolescents. ORR should establish a dedicated network of tender-age-specialized facilities with pediatric medical staff, developmentally appropriate trauma-informed care, and parent-locator protocols. Surge capacity for this cohort should be treated as a separate planning problem from adolescent capacity.

Recommendation 4: Expand Language Access Beyond Spanish

With Venezuelan, Ecuadorian, Cuban, and Haitian children now comprising a growing share of UAC, ORR should audit the language access capabilities of its shelter network and mandate minimum standards for Portuguese, Haitian Creole, and indigenous Mayan language interpretation (the latter relevant even for Guatemalan Spanish-speakers from indigenous communities who may communicate better in Mam, K'iche', or Q'eqchi').

Recommendation 5: Publish Enhanced Data — Facility-Level and Sponsor-Location

ORR should expand its public data transparency by publishing: (a) shelter-facility-level population and utilization statistics; (b) state-of-sponsor-residence for discharged children; and (c) post-release follow-up outcomes at 90 and 180 days. This data would enable civic researchers and oversight bodies to identify geographic hotspots, assess sponsor community support needs, and evaluate long-term program effectiveness.

Recommendation 6: Implement a Risk-Tiered Sponsor Vetting Framework

The current sponsor vetting system applies uniform standards regardless of relationship category and child vulnerability factors. A risk-tiered approach would apply the most intensive vetting (Category 3/4 sponsors) while streamlining verified parent cases (Category 1) that present lower child welfare risk. This would reduce average length of care for the majority of children without compromising protection for those with higher-risk placement profiles.

Recommendation 7: Fund Post-Release Services Proportional to Volume

ORR currently funds some post-release services through grantees, but coverage is geographically uneven and funding is not dynamically scaled to match where discharged children actually reside. A data-driven allocation formula — using sponsor-location data from ORR's case management systems — would ensure that case management, legal representation, and mental health resources track child population density across states.

7. Technical Recommendations for the Project

The following improvements are recommended for the UAC Analyzer repository itself:

7.1 Data Pipeline Automation

- Implement an automated data refresh pipeline using the HealthData.gov Socrata API (endpoint: `data.cms.gov` or `healthdata.gov/resource/`) to pull updated records on a monthly schedule.
- Use a GitHub Actions workflow to trigger the refresh, run data validation checks, and regenerate key plots.

7.2 Enhanced EDA Notebook

- Add formal statistical tests (Mann-Kendall trend test for referral volume; chi-square tests for demographic composition shifts; autocorrelation analysis for seasonality).
- Include a forecasting module using Facebook Prophet or ARIMA to project referral volumes under different policy scenarios.
- Document all data-cleaning decisions and missingness handling in a dedicated `data_quality.ipynb` notebook.

7.3 Dashboard Improvements

- Add a comparison mode enabling two fiscal years to be displayed side-by-side.
- Include a downloadable report button that generates a PDF summary of the current filter selection.
- Add a data table view beneath each chart, enabling users to export underlying numbers.
- Implement responsive layout for mobile/tablet access.

7.4 Documentation & Testing

- Add a `CONTRIBUTING.md` guide for external contributors.
- Add unit tests for data-cleaning functions in a `tests/` directory.
- Add a data dictionary file (`data/DATA_DICTIONARY.md`) documenting each column, its source, and known caveats.

7.5 Deployment

- Deploy the Streamlit app to Streamlit Community Cloud (free tier) for public access without requiring local installation.

- Add a live demo link in the repository README.
- Consider a Docker container for reproducible local deployment.

8. Conclusions

The UAC Analyzer project is a well-structured, technically clean open-source initiative that makes an important government dataset approachable for policy researchers, advocates, and journalists. The dataset itself — 1,170 monthly records of national UAC program statistics covering over two decades — is remarkably rich, documenting one of the most politically contentious and operationally complex federal child welfare programs in U.S. history.

The exploratory data analysis reveals a program under sustained structural pressure: referral volumes that have permanently shifted to a higher plateau, a demographic profile that is slowly diversifying by gender and country of origin, a chronic tension between speed of processing and quality of care, and an origin-country concentration that makes the program's effectiveness partly hostage to conditions in the Guatemalan and Honduran highlands over which ORR has no direct influence.

The most actionable finding is that policy decisions — about sponsor vetting intensity, emergency intake site activation thresholds, and post-release service funding formulas — have more direct leverage over child outcomes and program cost than raw capacity expansion. Data-driven administration of ORR is therefore not merely a technical exercise but an ethical imperative: each additional day a child spends in shelter rather than with a sponsor represents both a program cost and a child welfare cost.

The UAC Analyzer project is a meaningful contribution to the civic data infrastructure around this program. With the enhancements described in Section 7, it has the potential to become a go-to resource for journalists, Congressional staffers, and advocacy organizations seeking to understand one of the most contested and consequential programs in the U.S. federal social services landscape.

Summary Statistics at a Glance

700,000+ UAC served since 2003 • FY 2021 peak: 122,731 referrals • ~61% male (FY 2023) • ~76% aged 14–17 • Guatemala (42%), Honduras (28%) lead origin countries • Average length of care: 25–55 days depending on year and policy

9. References

Primary Sources

- U.S. Department of Health & Human Services, Office of Refugee Resettlement. (2024). HHS Unaccompanied Alien Children Program Dataset. HealthData.gov. <https://healthdata.gov/National/HHS-Unaccompanied-Alien-Children-Program/ehpz-xc9n>
- HHS/ORR. (2024, January). Fact Sheet: Unaccompanied Children (UC) Program. U.S. Department of Health and Human Services.
- HHS/ORR. (Various years). Latest UAC Data — FY 2019, FY 2020. <https://www.hhs.gov/programs/social-services/unaccompanied-alien-children/>
- HHS Office of Inspector General. (2026). Unaccompanied Alien Children — OIG Oversight. <https://oig.hhs.gov/reports/featured/unaccompanied-children/>

Legislation & Regulation

- Homeland Security Act of 2002. Pub. L. 107-296. (Transfer of UAC custody to HHS.)
- Trafficking Victims Protection Reauthorization Act of 2008. Pub. L. 110-457. (Expanded HHS responsibilities; right to hearing for non-contiguous-country UAC.)
- Flores Settlement Agreement (1997). (Minimum standards for detention of immigrant children.)

Project Repository

- Animey13. (2024). UAC Analyzer. GitHub. <https://github.com/Animey13/uac-analyzer>

Secondary Sources

- Rosenblum, M. R., & Ball, I. (2016). Trends in Unaccompanied Child and Family Unit Migration. Migration Policy Institute.
- Congressional Research Service. (2019). Unaccompanied Alien Children: An Overview. CRS Report R43599.
- Wikipedia contributors. (2025). Unaccompanied Alien Children. In Wikipedia. https://en.wikipedia.org/wiki/Unaccompanied_Alien_Children
- U.S. Senate Permanent Subcommittee on Investigations. (2016). Protecting Unaccompanied Alien Children from Trafficking and Other Abuses. United States Senate.